

When Mom Falls at 2 AM

The First 30 Minutes (and the 30 Mistakes Most Families Make)



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"I got the call at 11:47 PM. A daughter I'd never met. Her mom had fallen. She didn't know what to do."

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From Ryan

I got the call at 11:47 PM on a Tuesday. A woman I'd never met. Her name was Diane. Her mom had fallen in the bathroom. The neighbor had heard it through the wall and called Diane in Phoenix. Diane was crying. She lived three states away. She didn't know what to do first.

I'm not a paramedic. I'm not a doctor. I'm not a social worker. But for 8 years I bought houses from families exactly in Diane's situation, and I learned what mattered in the first 30 minutes versus what didn't. Then I switched sides. Now I help families avoid the mistakes I used to capitalize on.

This guide is the playbook I wish every family had taped to the inside of their fridge.

You won't read it cover to cover tonight. You'll read it the day before you need it. Or the day after. Either way, the goal is the same: when your phone rings at 2 AM, you don't have to figure it out from scratch.

Print this. Save it. Forward it to your siblings. The families who do best in crisis are the ones who decided what they'd do BEFORE the call came.

The First 5 Minutes

If you just got the call right now, read only this page. Come back for the rest later.

IF Mom is conscious and talking on the phone

Stay calm. Your voice is the only thing she has right now.

1. Ask: "Are you hurt anywhere? Can you move your arms and legs?"
2. Ask: "Where exactly are you? On the floor? Furniture nearby?"
3. Tell her: "Don't try to get up. I'm calling for help. Stay on the phone with me."
4. Hang up briefly to call 911 from another phone OR use 3-way calling
5. Tell 911: address, that she's fallen, age, conscious or not, any complaints of pain
6. Get back on the phone with Mom. Stay there until help arrives.

IF Mom is unconscious, in severe pain, hit her head, or you can't reach her

Call 911 FIRST. Right now. Don't call siblings. Don't call doctors. Don't call neighbors. Just 911.

Tell them:

- The address
- "My elderly mother has fallen and isn't responding" OR "is in severe pain"
- Her age
- Stay on the line — they'll dispatch immediately and keep you talking

What NOT to do in the first 5 minutes

- Do NOT tell her to "get up and walk it off"
- Do NOT call siblings before 911 (you can call them while you wait for paramedics)

- Do NOT drive over before help arrives if you live more than 10 minutes away
 - Do NOT try to talk her through getting up if she's hurt
 - Do NOT promise her "everything will be OK" — promise her "help is on the way"
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The First Hour

You've called 911 or confirmed Mom is stable. Now:

Call 1: Closest family member who can physically be there

In order of preference: spouse, sibling who lives near Mom, adult grandchild, trusted neighbor. Someone needs to be at Mom's house OR the hospital within an hour. That's not for emotional support. That's because:

- They need to make decisions for her until you arrive
- They need to grab her wallet, ID, insurance card, medication list, glasses
- They need to be able to give consent if she can't

Call 2: Yourself (if you can travel)

If she's been transported to the ER and you're more than a few hours away, make travel plans NOW. Don't wait to "see how she does overnight." Hospitals discharge fast. Decisions get made fast. You want to be physically present when they happen.

Book the flight before you call anyone else.

Call 3: Power of Attorney holder

If Mom has a Power of Attorney (POA), the holder needs to be reachable in the next hour. If you're the POA, you're up. If your sister is the POA, she needs to be reachable.

Don't have a POA in place? That's a problem you can't solve in the next hour. Skip to "What to Do BEFORE Crisis Hits" later. For now, the closest family member at the hospital can give verbal consent for emergency treatment.

Call 4: Her primary care doctor's after-hours line

Not for treatment — that's the ER's job. For records. Tell the answering service: "My mother [name, DOB] has been taken to [hospital]. Please notify Dr. [name] in the morning so they have the records ready when the hospital calls."

This single call shaves 24-48 hours off her hospital stay because it gets her existing care plan into the ER's hands faster.

What NOT to do in the first hour

- Do NOT post about it on Facebook (you'll get a flood of well-meaning calls that block your phone)
 - Do NOT promise siblings answers you don't have yet
 - Do NOT make any housing decisions
 - Do NOT agree to any "level of care" recommendations the ER suggests yet
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The First Day

Mom is at the ER. Paramedics took vitals. Maybe she's stable, maybe she's been admitted, maybe she's heading to imaging. Here's what to do in the first 24 hours.

The single most important question to ask: "Observation status or admitted status?"

These sound the same. They are not.

- Admitted: Medicare covers the hospital stay fully. Subsequent rehab/skilled nursing also covered (up to 100 days) IF she had 3+ consecutive midnights as admitted.
- Observation: Medicare treats this as outpatient. She can be in a hospital bed for 5 days under "observation" and still NOT qualify for Medicare rehab coverage when she's discharged.

This trap catches families with a \$15,000-\$50,000 surprise bill for rehab. Ask, in writing, "Is my mother's status admitted or observation?" Get the answer in writing. If they say observation and you can advocate to change it to admitted, do so.

Get the document she needs to keep

If you can find them, bring to the hospital or have a family member bring:

- Picture ID
- Medicare card / insurance cards
- Complete medication list (with doses)
- Power of Attorney (the document, not just the existence of one)
- Healthcare directive / living will
- Glasses, hearing aids, dentures, CPAP

If they're not findable in Mom's house, that's a separate problem worth solving in the next week. For now, get whatever you can.

Start asking about discharge planning IMMEDIATELY

Yes, on Day 1. Hospitals discharge fast. The person to ask is the case manager or social worker assigned to her. Their job is to plan her exit from the hospital, and they will start that plan whether you ask or not.

Questions to ask the case manager on Day 1:

- "What's the projected discharge date?"
- "What level of care will she need at discharge?"
- "What are her rehab options?"
- "What facilities in our area are in-network for her insurance?"

Decisions to NOT make on Day 1

- Selling her house
- Moving her into your house
- Putting her into a memory care facility
- Hiring 24/7 home care
- Anything that locks in a long-term plan

Day 1 decisions should be: hospital level of care, who's at the hospital, who's traveling there, who manages communication. That's it.

The First Week

Now things get complicated. Discharge is approaching. Decisions need to be made. Family dynamics get loud. Here's what matters.

Documents to track down or create

If Mom doesn't have current versions of these, they need to be created by the end of the week:

- Durable Power of Attorney (financial)
- Healthcare Power of Attorney
- Living Will / Advance Healthcare Directive
- HIPAA authorization (so doctors can share info with family)

This is attorney work. Most NC elder law attorneys can put together a basic package for \$400-\$800 in a single appointment.

Family meeting (in person or video)

Within 5 days of the fall, get the family on a video call. Not a group text. Not a group chain email. A real call. 30 minutes max. Cover:

- Medical update (one person briefs everyone)
- Discharge plan (what the hospital is recommending)
- Who's available for what (in-person care, financial, paperwork)
- Decision-making roles (who decides what, by when)

Don't try to decide HOUSING on this call. Decide who's authorized to research housing options. That's different.

Things to research this week (but not decide)

- Rehab facilities in her area (visit 2-3 if possible)

- In-home care options
- Adult day care options
- Senior housing communities
- Costs of each

You're collecting information, not committing to anything.

Decisions to absolutely NOT make in the first week

- Selling the house
- Signing her into long-term care
- Liquidating any of her financial accounts
- Cashing out her annuity / pension early
- Adding adult children to her bank accounts
- Major medication changes outside of doctor's orders
- Driving privileges (yes/no)

These all have legal, financial, and family consequences that need a slower decision. Day 7 is not the day.

The 5 Mistakes Families Make in Crisis Mode

These are the patterns I saw over and over for 8 years. Every single one is avoidable.

Mistake 1: Calling family before 911

Adult kids panic and call their sibling first to "ask what to do." 90 seconds later they realize they should have called 911. 90 seconds is real time in a stroke or cardiac event.

Rule: If there's ANY doubt about Mom's condition, 911 first. Family second.

Mistake 2: Making housing decisions inside the hospital

The hospital social worker tells you Mom needs a "higher level of care." A facility rep happens to be in the building. They offer a tour. By Day 3 Mom is signed into a place you've never visited and don't know the contract terms of.

Rule: Housing decisions happen AFTER discharge. Discharge can be to a temporary rehab, then home, then a real housing decision two weeks later. Don't lock in long-term housing during the crisis.

Mistake 3: Trusting the "free help" at the hospital

The discharge planner is paid by the hospital. Their job is to clear the bed, not to find the best long-term answer for Mom. The friendly senior placement person who shows up offering to help find a facility is usually paid commission by the facility they recommend.

Rule: Free advice from someone who profits when you say yes is not free. Always ask: "How are you compensated for helping us?"

Mistake 4: Not asking "observation or admitted?"

Already covered above. This one is the single biggest financial trap. Asking the question changes the financial outcome by \$15-50K.

Mistake 5: One sibling making all decisions alone

The closest sibling (geographically or emotionally) often takes over because they have to. Six months later, the other siblings are angry about decisions they weren't consulted on. The family fractures.

Rule: Even if one person is the legal decision maker, weekly check-in calls with all siblings prevent 80% of the future blow-up. Document decisions in writing (email works).

The Crisis Decision Framework

Use this when you don't know what to do.

Decisions that **MUST** be made in the first 24 hours

- Emergency medical treatment consent (POA or next of kin)
- Notify employer / arrange travel if needed
- Designate one person as the family point of contact for the hospital

Decisions that can wait 72 hours

- Family roles for the immediate care period
- Whether to engage an elder law attorney
- Communication plan for extended family

Decisions that should wait at least 2 weeks

- Whether Mom moves
- Whether to sell her house
- Long-term care provider selection
- Financial restructuring

Decisions that should wait at least 30 days

- Whether to move Mom into your house permanently
- Whether to take legal action (guardianship, conservatorship)
- Major changes to her existing professional relationships (doctor, lawyer, accountant)

When in doubt: the slower you can go, the better the decision usually is. Crisis brain optimizes for ending the discomfort. Calmer brain optimizes for the right outcome.

What to Do BEFORE Crisis Hits

If you're reading this and Mom hasn't fallen yet, you're in the best possible position. Spend a single Saturday doing the following and you'll be miles ahead of every family in your zip code when their crisis comes.

Paperwork to have NOW

- Durable Power of Attorney (financial)
- Healthcare Power of Attorney
- Healthcare directive / living will
- HIPAA authorization
- Updated will
- List of all her accounts, insurance, doctors, attorneys, accountants

Cost to set these up: \$400-\$800 with an elder law attorney. Worth every dollar.

A "If Mom Falls" plan in writing

One page. Include:

- Her address and gate code
- Her doctor's name and number
- Her medications (with doses)
- Her insurance numbers
- Closest hospital
- Who has POA
- Who's traveling first

Save it in a shared family Google Drive or Apple Notes. Update once a year.

Trial-run conversations with Mom

Awkward but important. Without panic, ask:

- "If you fell and couldn't get up, who do you want me to call first?"
- "If the doctor says you can't live alone anymore, where do you want to be?"
- "What kind of care do you NOT want, ever?"
- "Where do you keep your insurance card and medication list?"

Write down the answers. Confirm them with siblings.

A "decision pre-decided" with siblings

Before crisis hits, agree as siblings on:

- Who's the primary decision maker for medical
- Who's the primary decision maker for finances
- How decisions get communicated
- What requires unanimous family agreement vs single-person decision

This is the single most important pre-crisis investment. Family fights in crisis kill more relationships than the crisis itself.

The Bottom Line

When the call comes:

1. 911 before anything else if there's ANY doubt about Mom's condition
2. The hospital is a starting point, not a destination — don't make long-term decisions there
3. Ask "observation or admitted?" — write down the answer
4. One person at the bedside, one person on the phone, others in research mode — divide the labor
5. No housing decisions for at least 2 weeks — short-term rehab buys time

Crisis brain wants to end the discomfort. Calmer brain wants the right outcome. Your job in the first 30 minutes is to keep crisis brain from making decisions only calmer brain should make.

You will not handle this perfectly. Nobody does. The goal isn't perfect. The goal is to avoid the mistakes that cost \$50,000 or fracture the family.

This guide is yours. Forward it to your siblings. Print it and tape it to the fridge. The families who do best are the ones who decided what they'd do BEFORE the call came.

What's Next

You have free access to the Senior Transition Blueprint dashboard. Inside:

- Module 00: Foundations and Quick Start — How to think about senior transitions before the crisis hits
- The 7-Day Quick Start Checklist — Day-by-day actions for the first week after a transition triggers
- The Net Proceeds Calculator — Compare housing options with real numbers
- The Pre-Sale Quiz — Find out which of the 5 selling strategies fits if a housing change is needed

If you want help applying any of this to your specific family situation, the Blueprint Premium (\$297) includes a 60-minute call with me. We walk through your situation, I tell you what I would do, you take notes. No follow-up pitch.

If you just want the tools and you've got it from here, the Blueprint Core (\$47) gives you the full 19-module course. Read at your own pace.

Either way, this guide is yours to keep. Forward it. Print it. Save it for when you need it. The families who walk into crisis prepared are the ones who walk out of it intact.

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This guide is not medical, legal, or financial advice. Every family's situation is different. For specific medical questions, call 911 or your doctor. For specific legal questions, consult a licensed elder law attorney in your state.

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